GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Nazeera Farook
Patient number	PAT-2026-00010
DOB	2004-08-22
Sex	female
Assessment ID	8915d2d9-91ce-4e6e-87aa-958f6720ae23
Assessment time	2026-10-15 10:40:50.494525
Hospital	Durdans Hospital
Department	Not recorded
Clinician / operator	Local Super Admin
Model	GOVCARE-CKD-LOGISTIC-1.0

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
98.7%	**URGENT REVIEW**	**97.4%**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Stage 5
G category	G5
Suggested ICD code	N18.5
Clinical description	Chronic kidney disease, stage 5
Terminology system	http://hl7.org/fhir/sid/icd-10-cm
Basis	Reviewed eGFR 4 mL/min/1.73m²; chronicity and diagnosis require clinician confirmation
Coding status	AI_ASSISTED_SUGGESTION
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	○ G2	○ G3a	○ G3b	○ G4	● G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	4	mL/min/1.73m²	MANUAL
Serum creatinine	3	mg/dL	MANUAL
Urine albumin/creatinine ratio (ACR)	4	mg/g	MANUAL
Urine protein	4	mg/dL	MANUAL
Systolic blood pressure	241	mmHg	MANUAL

Diastolic blood pressure	41	mmHg	MANUAL
HbA1c	4	%	MANUAL
BMI	41	kg/m²	MANUAL
Serum potassium	4	mmol/L	MANUAL
Hemoglobin	4	g/dL	MANUAL

CKD Related Clinical Intelligence Summary

Source evidence, monitoring status and next clinical actions

Assessment-linked snapshot: 9ed151c1-8926-4003-901d-13775fae5e48 | Evaluated: 2026-10-15T05:10:50.816270400Z

Anemia Monitoring

Risk status: Source review recommended

Current Hb: 4 g/dL | Undated snapshot; verify date

Clinical Data Readiness: 100% Ready | Available evidence: 1/1 monitoring inputs

Clinical indicators: Hemoglobin 4 g/dL

Missing information: Listed monitoring inputs recorded

Extended workup: PCV, RBC count, ferritin and iron profile are not linked to this snapshot; review CBC / iron-study sources.

AI explanation / clinical rule: Hemoglobin 4.00 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.

Research prediction: Eligible dialysis context not verified

Clinical action: Add historical Hb result

Clinician notes: No notes stored for this module snapshot.

Hyperkalemia Risk Monitoring

Risk status: Verify measurement date

Current potassium: 4 mmol/L | Undated snapshot; verify date

Clinical Data Readiness: 100% Ready | Available evidence: 1/1 monitoring inputs

Clinical indicators: Serum Potassium 4 mmol/L

Missing information: Listed monitoring inputs recorded

AI explanation / clinical rule: Serum potassium 4.00 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.

Research prediction: Eligible dialysis context not verified

Clinical action: Add historical potassium result

Clinician notes: No notes stored for this module snapshot.

Dialysis Effectiveness Monitoring

Risk status: Data required

URR: Not calculated

Clinical Data Readiness: 0% Ready | Available evidence: 0/3 monitoring inputs

Clinical indicators: Source measurements required

Missing information: Pre-HD Urea; Post-HD Urea; Dialysis date

AI explanation / clinical rule: Insufficient source evidence for interpretation.

URR = ((Pre-HD urea - Post-HD urea) / Pre-HD urea) x 100

Research prediction: Eligible dialysis context not verified

Clinical action: Add dialysis-session data

Clinician notes: No notes stored for this module snapshot.

Renal Function Progression

Risk status: Verify measurement date

Clinical Data Readiness: 25% Ready | Available evidence: 1/4 monitoring inputs

Clinical indicators: Serum Creatinine 3 mg/dL

Missing information: Previous creatinine; Current creatinine measurement date; Previous creatinine measurement date

AI explanation / clinical rule: Insufficient source evidence for interpretation.

Clinical action: Add baseline creatinine history

Clinician notes: No notes stored for this module snapshot.

CKD Cardiovascular Monitoring

Risk status: Surveillance recommended | Risk not estimated

Clinical Data Readiness: 80% Ready | Available evidence: 4/5 monitoring inputs

Clinical indicators: Systolic Blood Pressure 241 mmHg; Diastolic Blood Pressure 41 mmHg; Serum Potassium 4 mmol/L; Serum Creatinine 3 mg/dL; Age 22 years

Missing information: Clinical diabetes status

AI explanation / clinical rule: The dashboard presents actual blood pressure, potassium, creatinine and age when available, with their source dates.

Prediction readiness: cardiovascular model unavailable; factor completeness is not a risk probability.

Clinical action: Review cardiovascular factors

Clinician notes: Review / record findings in the existing patient clinical record.

Why this result?

Interpret recorded creatinine, eGFR, potassium and hemoglobin with their actual source dates. Arrows and trends require repeated dated observations. Missing values do not imply low risk. Clinical monitoring uses transparent existing measurement rules. Research forecasts remain separately gated. Detailed source evidence, model information, limitations and stored clinician reviews follow.

Data completeness is not diagnostic certainty, forecast eligibility or disease probability. No inference is run during export.

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: 9ed151c1-8926-4003-901d-13775fae5e48 | Evaluated 2026-10-15T05:10:50.816270400Z. Export reads saved clinical evidence; no inference is performed.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

Prediction generated from available clinical measurements. Longitudinal forecasting requires repeated dated observations. Clinical data readiness is input completeness, not diagnostic certainty or disease probability.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

CKD Progression / Renal Trajectory

CALCULATED - High CKD progression monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Transparent snapshot monitoring rule

Qualitative measurement-based monitoring band; no future-event probability or forecast horizon is estimated.

Single clinical snapshot available. Longitudinal trend requires previous dated observations.

Clinical Data Readiness: 100% Ready | 9/9 parameters | Missing: 0

GovCare qualitative eGFR/ACR grid: High if eGFR <45, ACR >300, or eGFR <60 with ACR ≥30; Moderate if eGFR <60 or ACR ≥30; Low requires eGFR ≥60 AND ACR <30. Categories condense high/very-high monitoring; not a validated progression model.

Serum Creatinine: 3 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

eGFR: 4 mL/min/1.73m² | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Protein in urine: 4 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Urine ACR: 4 mg/g | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Age: 22 years | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Systolic Blood Pressure: 241 mmHg | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Diastolic Blood Pressure: 41 mmHg | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Fasting Blood Sugar: 141 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

HbA1c: 4 % | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Clinical recommendations

Clinician review of eGFR, ACR, urine findings and renal chronicity; obtain previous dated observations for longitudinal forecasting.

Why: Reduced eGFR: 4.00 mL/min/1.73 m2.

Why: ACR: 4.00 mg/g.

Why: Serum creatinine: 3.00 mg/dL; interpret against the patient's source reference range.

Why: HbA1c / glucose are supporting glycemic measurements; a diabetes diagnosis is not inferred.

Available: Serum Creatinine | 1/1 | 3.00 mg/dL; undated snapshot only

Available: eGFR | 1/1 | 4.00 mL/min/1.73m²; undated snapshot only

Available: Protein in urine | 1/1 | 4.00 mg/dL; undated snapshot only

Available: Urine ACR | 1/1 | 4.00 mg/g; undated snapshot only

Available: Age | 1/1 | 22.00 years; undated snapshot only

Available: Systolic Blood Pressure | 1/1 | 241.00 mmHg; undated snapshot only

Available: Diastolic Blood Pressure | 1/1 | 41.00 mmHg; undated snapshot only

Available: Fasting Blood Sugar | 1/1 | 141.00 mg/dL; undated snapshot only

Available: HbA1c | 1/1 | 4.00 %; undated snapshot only

Next action: Add historical laboratory data

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.

Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

CALCULATED - High kidney failure monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Clinical evidence / monitoring rule

Kidney failure risk indicator; clinical review required. This does not confirm a diagnosis or establish G5D.

Dialysis verification required

Clinical Data Readiness: 80% Ready | 4/5 parameters | Missing: 1

eGFR: 4 mL/min/1.73m² | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Serum Creatinine: 3 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Protein in urine: 4 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Urine ACR: 4 mg/g | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Clinical recommendations

Review renal function and chronicity; verify dialysis status before G5D labeling or maintenance-HD model use.

Why: eGFR 4.00 mL/min/1.73 m2 is below the 15 monitoring threshold.

Available: eGFR | 1/1 | 4.00 mL/min/1.73m²; undated snapshot only

Available: Serum Creatinine | 1/1 | 3.00 mg/dL; undated snapshot only

Available: Protein in urine | 1/1 | 4.00 mg/dL; undated snapshot only

Available: Urine ACR | 1/1 | 4.00 mg/g; undated snapshot only

Needs review / data: Verified dialysis status | 0/1 | Dialysis verification required

Next action: Verify dialysis status

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.

No 2-year or 5-year kidney-failure probability is calculated.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

CALCULATED - Below hyperkalemia monitoring threshold

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current potassium monitoring rule

Current measurement monitoring is independent of the maintenance-HD research forecast. The low band does not exclude hypokalemia or other abnormalities.
Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0
Serum Potassium: 4 mmol/L | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Clinical recommendations

Review measured potassium with the clinician, source quality, medications and sampling context; use true paired pre/post-HD samples when available.
Why: Serum potassium 4.00 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.
Available: Serum Potassium | 1/1 | 4.00 mmol/L; undated snapshot only
Next action: Add historical potassium result
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

CALCULATED - Anemia monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current hemoglobin monitoring rule
Measured Hb monitoring and decline rate are separate from the installed persistence baseline; the baseline is not an anemia prognosis.
Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0
Hemoglobin: 4 g/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Clinical recommendations

Clinician review against age/sex-specific laboratory ranges, iron studies and clinical context; add a previous dated Hb result for deterioration monitoring.
Why: Hemoglobin 4.00 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.
Available: Hemoglobin | 1/1 | 4.00 g/dL; undated snapshot only
Next action: Add historical Hb result
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

INSUFFICIENT DATA - Dialysis session measurements required

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Measured same-session urea reduction ratio
Requires hemodialysis session data
Clinical Data Readiness: 0% Ready | 0/3 parameters | Missing: 3
URR = ((Pre-HD Urea − Post-HD Urea) / Pre-HD Urea) × 100

Clinical recommendations

Review measured adequacy with the clinician; record actual pre-HD urea, post-HD urea and dialysis date. BUN from the CKD form is not a paired dialysis urea measurement.
Needs review / data: Pre-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Post-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Dialysis date | 0/1 | Actual dialysis session date; upload time is not substituted
Next action: Add dialysis-session data
Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 0 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

AKI / Renal Clinical Deterioration

PARTIALLY CALCULATED - AKI trend not calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Dated creatinine change / acute review rule
Baseline creatinine required for AKI trend calculation
Clinical Data Readiness: 25% Ready | 1/4 parameters | Missing: 3
Creatinine-based clinical review only: rise ≥0.3 mg/dL within 48 h or ≥1.5 times a dated baseline within 7 days. No AKI staging, urine-output assessment or diagnosis is generated.
Creatinine change (%) = ((Current Creatinine − Previous Creatinine) / Previous Creatinine) × 100
Serum Creatinine: 3 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 8915d2d9-91ce-4e6e-87aa-958f6720ae23

Clinical recommendations

Obtain previous creatinine with its actual measurement date; clinician review of timing, acute illness, medications and urine output is required.
Available: Serum Creatinine | 1/1 | 3.00 mg/dL; undated snapshot only
Needs review / data: Previous creatinine | 0/1 | Baseline creatinine required for AKI trend calculation
Needs review / data: Current creatinine measurement date | 0/1 | Current and previous results need distinct actual dates
Needs review / data: Previous creatinine measurement date | 0/1 | A recorded baseline date is required

Next action: Add baseline creatinine history

Directional composite – not a validated prognosis score.

Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero.

A directional percentage is not a disease probability.

Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

PREDICTIVE CLINICAL INTELLIGENCE - LONGITUDINAL REVIEW

Live authorized EHR history at report generation time. Projected values are statistical extrapolations and are never stored or presented as laboratory observations.

Recorded G / A category	Latest eGFR	Latest creatinine	Data completeness
- / -	Unavailable	Unavailable	Unavailable

HISTORICAL eGFR & PROJECTED TRAJECTORY

No usable dated eGFR history is available.

Horizon	Current	Projected	Projected range	Trend	Reliability	Status / interval method
3 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
6 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
12 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED

EXPLAINABILITY & DATA-QUALITY LIMITATIONS

Renal safety: INSUFFICIENT DATA. Renal observations have unresolved date, unit, provenance, plausibility or conflicting-result issues. Reconcile the source records before forecasting. A usable eGFR result from the previous 90 days is unavailable. A usable creatinine result from the previous 90 days is unavailable. At least two dated eGFR measurements spanning 90 days are required by the longitudinal sufficiency policy.
- MISSING CLINICAL DATE: A usable measurement date is required; upload time is never substituted.
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Medication context: Recent active prescription records provide context only. Actual use, adherence, stop dates, non-prescription drugs and acute illness/fluid status are not confirmed. Reconcile with the patient.

CLINICIAN-CONTROLLED RECOMMENDATIONS

- No dated eGFR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.
- No dated Urine ACR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.

Method: TREND PROJECTION · Model: trend-linear-2.0 · Validation: STATISTICAL_PROJECTION_NOT_CLINICALLY_VALIDATED · Local validation: NOT_APPLICABLE

Clinical decision support only. Forecasts and risk estimates are not autonomous diagnoses.

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding
Urine albumin/creatinine ratio (ACR)	4	mg/g	MANUAL	-

Age	22	-	CALCULATED	-
BMI	41	kg/m²	MANUAL	-
Blood urea nitrogen (BUN)	41	mg/dL	MANUAL	-
Diastolic blood pressure	41	mmHg	MANUAL	-
Edema	1	-	MANUAL	-
Family history of kidney disease	1	-	MANUAL	-
Fasting blood sugar	141	mg/dL	MANUAL	-
eGFR	4	mL/min/1.73m²	MANUAL	-
HbA1c	4	%	MANUAL	-
Hemoglobin	4	g/dL	MANUAL	-
NSAID use	1	-	MANUAL	-
Previous acute kidney injury	1	-	MANUAL	-
Urine protein	4	mg/dL	MANUAL	-
Serum creatinine	3	mg/dL	MANUAL	-
Serum potassium	4	mmol/L	MANUAL	-
Systolic blood pressure	241	mmHg	MANUAL	-

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution
Systolic blood pressure	241	mmHg	Increases prediction	1.6174
eGFR	4	mL/min/1.73m²	Increases prediction	1.4332
Hemoglobin	4	g/dL	Reduces prediction	-0.8762
Edema	1	-	Increases prediction	0.5688
Diastolic blood pressure	41	mmHg	Reduces prediction	-0.5454
Urine protein	4	mg/dL	Increases prediction	0.4194
HbA1c	4	%	Reduces prediction	-0.4187
Blood urea nitrogen (BUN)	41	mg/dL	Increases prediction	0.3887
BMI	41	kg/m²	Increases prediction	0.3385
Family history of kidney disease	1	-	Increases prediction	0.3341
Urine albumin/creatinine ratio (ACR)	4	mg/g	Increases prediction	0.1993
Serum creatinine	3	mg/dL	Increases prediction	0.1499

RECOMMENDED CLINICAL REVIEW

1. Prompt clinician review is recommended.
2. Correlate with current renal function, urine findings, and clinical examination.

CLINICIAN REVIEW

Suggested classification	Stage 5
Suggested code	N18.5
Coding decision	[ ] Confirm [ ] Modify [ ] Reject

Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.

FIVE-YEAR CKD OUTLOOK

Future risk, current evidence, and clinical planning

RESEARCH PREVIEW | Local clinical validation: NOT COMPLETED

No long-term outlook assessment has been stored. Open the authorized 5-Year CKD Outlook workspace to review eligibility and calculate an assessment. No new inference or risk calculation was performed for this export.